LUMINA

Understanding the PMR Bid

From Pharma RFP to Study Execution — A Complete Field Guide

Includes: How a PMR Bid Differs • The Bid Process Step-by-Step • Survey vs Interview • HCP Sourcing • How to Win • Competitor Landscape

Audience	Purpose	Date
All team members — especially those new to healthcare PMR	**Build shared understanding of the market Lumina serves**	**March 2026**

PART 1 — Software RFP vs PMR Bid Why They Are Completely Different

You're familiar with software RFPs. A PMR bid looks superficially similar — a client sends questions, vendors respond — but the similarity ends there. Here's why:

Dimension	Software / IT RFP (what you know)	PMR Bid (what Lumina serves)
What is being procured	A product or system that will be built or licensed	A custom research study that will be designed, executed, and analysed — unique every time
What the client is buying	A defined capability (features, integrations, SLAs)	An answer to a business question (e.g. 'will doctors prescribe our new drug?')
How complex is the brief?	Usually structured — specs, requirements, architecture questions	Often vague — 'understand prescribing behaviour in oncology across 5 markets'
Who you're buying from	A software vendor with a product	A research firm that will design a bespoke study methodology from scratch
What the proposal contains	Feature list, pricing tiers, implementation plan, SLA	Research design, sample plan, HCP list, survey/discussion guide, timeline, costs, legal contracts
How long it takes to respond	Days to weeks	2 days to 2 weeks depending on complexity
Human expertise involved	Sales + pre-sales + solution architect	Bid lead + research director + fieldwork ops + finance + legal — all in parallel
What happens after you win	You build / deliver the software	You actually run the study — recruit doctors, conduct interviews or surveys, analyse results, present findings

Dimension	Software / IT RFP (what you know)	PMR Bid (what Lumina serves)
Can you re-use the proposal?	Similar RFPs get similar proposals	Every study is unique — methodology, sample, markets, analysis all vary
Compliance complexity	Data security, GDPR	GDPR + HCP transparency laws + fair market value (doctor incentives) + adverse event reporting + patient privacy

The key insight

In a software RFP, you are selling a product. In a PMR bid, you are selling your expertise and your plan for answering a scientific question. The client is trusting you with their most commercially sensitive decisions — drug pricing, launch strategy, market access. The stakes are high, and that is why the process is complex.

PART 2 — What Questions Does a PMR RFP Actually Ask?

When a pharma company sends a PMR RFP, they are asking a research firm to answer a business question. But the RFP itself contains two levels of questions: the business question they want answered, and the operational questions about how you will answer it.

Level 1 — The Business Question (What the Pharma Client Wants to Know)

These are the questions the pharma company is trying to answer through the research. Examples:

Research objective type	Example business question from pharma	Study type typically used
Understanding & Attitudes	Do oncologists understand the mechanism of action of our new drug? What are their current prescribing barriers?	U&A study (quant survey, 200–400 HCPs)
Concept / Messaging Test	Which of these 3 product positioning messages resonates most strongly with cardiologists?	Concept test (quant, 150–300 HCPs)
Pricing / Willingness to Pay	At what price point would payers in Germany, France, and UK reject this drug? What evidence do they need?	Payer IDI research + WTP modelling
Segmentation	Can we identify distinct 'types' of neurologists by their prescribing philosophy? How big is each segment?	Segmentation build (quant, 500–800 HCPs)
Tracker / Monitoring	Is awareness of our drug increasing among GPs quarter on quarter? Is the message landing?	Brand tracker (quant, repeated waves, 100–200/wave)
Needs & Barriers (qual)	Why do rheumatologists hesitate to prescribe biologics even when patients are eligible?	In-depth interviews (IDIs), 20–40 sessions
Market Access / HTA	What evidence do HTA bodies in 5 markets need before they will approve reimbursement?	Payer/HTA IDI research, 15–30 sessions

Research objective type	Example business question from pharma	Study type typically used
Journey Mapping	What is the full patient journey from first symptoms to treatment decision in Type 2 diabetes?	Qual deep dives — IDI + diary studies

Level 2 — The Operational Questions in the RFP (What the Firm Must Answer)

The RFP document itself will ask the research firm to respond to questions like these. This is what the bid must address:

Section A: Understanding of the Brief

- Can you restate our research objectives in your own words?
- What do you see as the core business decision this research will inform?
- What assumptions are you making where we haven't been specific?

Section B: Research Design (the most scrutinised section)

- What methodology are you recommending — quantitative survey, qualitative interviews, or a mix? Why?
- How many respondents are you proposing, broken down by country and audience segment?
- What is your rationale for this sample size — statistical power? Quota requirements? Feasibility?
- What data collection mode will you use — online (CAWI), telephone (CATI), face-to-face, video IDI?
- How long will the survey/interview be (Length of Interview, LOI)?
- What advanced analytics are you proposing — drivers analysis, conjoint, MaxDiff, segmentation?

Section C: Audience & Recruitment (critical and often underdeveloped by weaker firms)

- Who exactly will you recruit — specialty, practice setting, patient volume thresholds, country?
- How will you recruit them — panel, your own proprietary database, recruiter, sites/clinics?
- What are your inclusion and exclusion criteria?
- How will you ensure the HCPs you recruit are actually qualified and not survey fraudsters?
- What are the incentives you will pay HCPs, and how do they comply with fair market value rules?
- What is your feasibility estimate — how many eligible HCPs exist in your database for this spec?

Section D: Instruments (the survey or discussion guide)

- Will you provide a draft survey questionnaire / discussion guide as part of the bid?
- How many questions / modules are you proposing?
- Will you include stimuli (concept boards, positioning statements, pricing scenarios)?
- Who will programme the survey (if quant)? What platform?

Section E: Timeline

- When can you start? When will fieldwork begin and end?

- When will analysis be complete? When will you deliver the topline? The full report?
- How many client review windows are built into the timeline?
- Can you meet our required delivery date?

Section F: Deliverables

- What will we receive at the end — a PowerPoint deck, a data tabbook, raw data, transcripts?
- Will there be a readout/presentation session?
- Will you provide an executive summary / topline before the full report?

Section G: Pricing

- What is your total cost, broken down by phase (design, fieldwork, analysis, reporting)?
- What is included and what is out of scope?
- What are your payment terms?
- Can you offer options (e.g. reduced sample = lower cost)?

Section H: Team & Credentials

- Who will lead this project? What is their experience in this therapy area?
- Who will do the analysis and moderation?
- Can you provide case studies of similar work?
- Are you ESOMAR / MRS / BHBIA compliant?

PART 3 — The End-to-End PMR Bid Process Step by Step

Here is what actually happens inside a PMR firm from the moment the RFP arrives to the moment the bid is submitted. This is the process Lumina automates.

1

RFP ARRIVES

Who: Bid Desk / Bid Lead

Client sends an email (or portal upload) with the RFP document and sometimes attachments — previous study reports, a protocol, a target audience spec.

- Read and classify: what type of study is this? (U&A, tracker, concept test, qual, etc.)
- Identify decision: is this worth bidding? (strategic fit, margin potential, team capacity)
- Check the deadline — PMR bids are often due in 48-72 hours
- Assign a Bid Lead who owns the response end-to-end
- Create the bid file — one folder/system for all bid artefacts

- Flag if clarifications are needed immediately (before any work starts)

2

CLARIFICATIONS SENT TO CLIENT

Who: Bid Lead

Almost every RFP has gaps. The firm sends a short list of questions to the client — professionally, quickly. The goal is to get missing information without sounding incompetent.

- What is the primary decision this research will inform? (if not stated)
- What is your target audience spec — specialty, country, patient volume, setting?
- What is your preferred survey length / number of IDIs?
- Do you have existing stimuli or are we designing them from scratch?
- What is your budget range? (if not stated — often they won't say)
- Are there any compliance constraints — GDPR, adverse event process, restricted topics?
- What is your preferred fieldwork window? Your hard deadline?
- Key rule: ask everything in ONE email. Never go back twice. It looks disorganised.

3

RESEARCH DESIGN

Who: Research Director / Senior Researcher

This is the intellectual core of the bid. A senior researcher decides HOW the study will be conducted. This decision determines everything downstream — cost, timeline, team.

- Choose methodology: quant (survey), qual (interviews/groups), or mixed
- QUANT: define sample size per country per audience segment. Justify statistically or by precedent.
- QUAL: define number of sessions (IDIs or FGDs), length, markets
- Choose mode: CAWI (online survey), CATI (telephone), video IDI, face-to-face IDI, FGD
- Define audience precisely: e.g. 'Oncologists treating >= 5 metastatic breast cancer patients/month in hospital settings, US/DE/JP'
- Recommend advanced analytics if appropriate: conjoint, MaxDiff, drivers, segmentation
- Draft the scope narrative: a plain English description of what will be done and why

- THIS IS WHAT CLIENTS SCRUTINISE MOST — weak methodology = losing the bid

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SAMPLE PLANNING & FEASIBILITY CHECK

Who: Fieldwork / Operations Team

Before pricing anything, ops must confirm that the target audience actually exists in sufficient numbers and can be recruited within the timeframe. This step prevents over-promising.

- Run a universe query: how many doctors matching the spec exist in your database/network?
- E.g. 'Oncologists, US, treating >= 5 mBC patients/month, hospital setting' → query finds 1,240 eligible
- Assess incidence rate: of those, what % will actually respond? (HCPs are hard to reach — often 5-15%)
- For 200 completes at 10% response rate → need to contact 2,000 eligible HCPs
- Flag if the spec is too narrow ('rare disease oncologist in Luxembourg' = near impossible)
- Propose backup options: broaden spec, extend timeline, use recruiter, increase incentive
- Create the sample plan: country x audience x n (number of completes) grid
- Create HCP shortlist: primary pool of names/IDs to contact, plus backup pool
- This step IS disclosed in the bid — but at the universe/count level, not individual names

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WORKPLAN & TIMELINE BUILD

Who: Project Manager

Build a realistic timeline from today's date to final delivery. Every milestone must account for client review time — the most common source of delay.

- Kickoff call with client (1-2 days after win)
- Research instrument design (survey/DG draft): 5-10 days
- Client review of instrument: 3-5 days (client review time is often underestimated)
- Survey programming / recruiter briefing: 3-5 days
- Soft launch / pilot: 2-3 days
- Full fieldwork: 2-6 weeks depending on audience difficulty and n

- Data processing / DP: 3-5 days (quant); transcription: 5-7 days (qual)
- Analysis: 7-14 days
- Report writing: 5-10 days
- Client review of report: 3-5 days
- Final delivery + readout session
- Total typical timeline: 8-16 weeks depending on complexity

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WBS & EFFORT ESTIMATION

Who: Bid Lead + Senior Researcher

Break every task into hours, per role. This is the most time-consuming manual step — and the biggest source of inconsistency between bid leads. This is what Lumina's Estimation Engine replaces.

- List every task: instrument design, programming, fieldwork management, data processing, coding (qual), analysis, charting, report writing, QA, PM, legal review...
- Assign hours per role to each task: Research Director, Senior Researcher, Analyst, Programmer, Fieldwork Manager, PM
- Apply complexity multipliers: more countries = more PM hours; longer survey = more programming hours; rare audience = more fieldwork ops time
- Sum hours per role to get total effort
- Identify out-of-pocket (OOP) costs: panel fees, HCP incentives, translation, transcription, facility hire, software
- Double-check against comparable past studies if available
- Produce a Work Breakdown Structure (WBS) — the granular task list with hours

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PRICING PACK BUILD

Who: Bid Lead + Finance

Apply rate cards to hours. Add OOP. Check margins. Get approvals. Build the final commercial offer.

- Labour cost: hours per role × cost rate per hour
- Labour price (what client pays): hours per role × sell rate per hour
- OOP: incentives (e.g. $150/HCP × 200 HCPs = $30,000), panel fees, translation ($800/language), transcription ($2/minute × 40 hours), etc.
- Total price = labour price + OOP

- Gross margin check: (total price - total cost) / total price — must exceed minimum threshold
- If margin is too low: check if hours are inflated, OOP can be reduced, or senior approval needed for exception
- Build options: Good (minimum sample), Better (recommended), Best (high confidence n) — each with cost/timeline impact
- Finance approver signs off if discount or margin exception is applied

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PROPOSAL DOCUMENT DRAFTING

Who: Research Lead + Bid Lead

Write the actual proposal document. This is the client-facing artefact — professional, branded, persuasive. Typically 15-40 pages.

- Cover page and executive summary: restate the brief, confirm you understand the objective
- Research approach: methodology, rationale, design choices — the 'why this approach' section
- Sample plan: who, how many, which countries, recruitment approach, feasibility statement
- Timeline: gantt-style or milestone table with key dates
- Deliverables: what they will receive and when
- Team page: who leads the study, their credentials, relevant experience
- Credentials / case studies: 1-2 relevant past studies (anonymised client, real outcomes)
- Commercial: total price, payment schedule, what's in/out of scope
- Assumptions and exclusions: the legally important section — 'price is based on these assumptions'
- Statement of Work (SoW): the legal contract document, often a separate attachment
- Pricing annex: detailed cost breakdown (may be internal-only or shared with client)

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INTERNAL APPROVALS

Who: Finance + Legal + Compliance

Before anything goes to the client, internal gates must be passed. Most firms skip this step manually — Lumina makes it enforced and logged.

- Scope approval: Research Director confirms the design is sound and deliverable

- Pricing/margin approval: Finance confirms margin is above threshold, discount is within policy
- Legal approval: Legal reviews the SoW for liability clauses, IP ownership, data handling
- Compliance approval: Compliance checks HCP consent language, adverse event process, GDPR statements
- If rejected at any gate: bid returns to the relevant section for rework, version increments
- All approvals are logged with timestamp and decision — critical for audit if a dispute arises later

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SUBMISSION

Who: Bid Lead

The completed bid package is assembled and sent to the client. This sounds simple — but sending the wrong version or missing an attachment has lost real bids.

- Final bid package: Proposal (PDF/Word), Pricing annex (Excel), SoW (Word/PDF), any appendices
- Name files correctly: [ClientName]_[ProjectTitle]_Proposal_v2_FINAL.pdf
- Write the covering email: brief, professional, summarises the key offer in 3-4 sentences
- If submitting via portal: check file size limits, required formats, deadline time (often 5pm local time!)
- Log submission: what was sent, when, to whom, which versions
- Confirm receipt if possible
- Brief the team that a bid has been submitted so everyone is ready for the next step

PART 4 — Survey vs Interview When Do You Use Each?

This is one of the most fundamental decisions in research design — and clients often ask firms to justify their choice. Here is the full logic:

Factor	Quantitative Survey (CAWI/CATI)	Qualitative Interviews (IDI/FGD)
Purpose	Measure, count, track — 'how many' and 'how much'	Understand, explore — 'why' and 'how does it feel'
Business question	'What % of oncologists are aware of Drug X?'	'Why do oncologists hesitate to prescribe Drug X?'
Sample size	100–800+ HCPs	10–40 IDIs or 4–8 FGDs (6-8 per group)
Length	10–35 minute online survey	45–90 minute video/telephone interview

Factor	Quantitative Survey (CAWI/CATI)	Qualitative Interviews (IDI/FGD)
Data type	Numerical (scales, ratings, choices, quotas)	Verbal (transcripts, quotes, themes)
Analysis	Statistical: means, significance testing, drivers, segmentation	Thematic: coding, pattern identification, insight synthesis
Deliverables	Tabbook (cross-tabs), top-line deck, full report	Insight report, quote bank, workshop, journey map
Cost	Lower per respondent; higher total due to large n	Higher per respondent (recruiter, incentive, moderator); smaller n
Stimulus testing	Strong — can show concepts to large groups simultaneously	Stronger — can probe reactions in depth, uncover nuance
Typical use in pharma	Brand tracking, KPI monitoring, concept testing, segmentation, WTP	Needs & barriers, patient journey, HTA payer research, early-stage concept exploration

Mixed methodology

For complex briefs, firms often propose a 'mixed' or 'sequential' design: qual first (IDIs to understand the landscape, identify key themes), then quant (survey to measure and size those themes across a larger sample). This is more expensive but gives the richest answer. It must be explicitly justified in the bid with a clear rationale for each phase.

Do You Need to Submit a Draft Survey in the Bid?

This is a common question. The answer: it depends on the client and the study type, but here is the general rule:

What to include in the bid	When	Why
Proposed module list / topic guide outline	Always — for both quant and qual	Shows you understand what needs to be covered; takes 1-2 hours to produce; high impact on client confidence
Draft survey structure (sections, number of questions per section)	For quant bids — strongly recommended	Clients want to see LOI is realistic; shows programming complexity is accounted for in cost
Full draft questionnaire	For large, complex quant bids — or if client explicitly asks	Differentiates you from competitors; but time-intensive — only invest if win probability is high
Discussion guide outline	For qual bids — always recommended	Moderator quality and guide structure is a key selection criterion for qual work
Full stimulus concepts	Almost never at bid stage	Stimulus is designed after the win — too expensive to develop speculatively

PART 5 — Where Do HCPs Come From? The Recruitment Question

This is one of the biggest operational differentiators between PMR firms. Getting 200 specialist cardiologists to complete a 20-minute survey is genuinely hard. Here are all the ways it is done:

Source 1: Proprietary Panel / Internal Database (the gold standard)

The best firms have spent years building their own database of verified, opted-in HCPs who have agreed to participate in research. This is the most valuable asset a PMR firm owns.

- HCPs are pre-screened, verified (licence check, specialty confirmation, patient volume), and profiled
- Response rates are higher — 15-30% vs 5-10% for cold outreach
- Turnaround is faster — no need for a third-party recruiter
- Compliance is cleaner — consent obtained at registration, fair market value rates pre-agreed
- For Lumina: the HCP shortlist feature queries this database and generates a primary + backup pool

Source 2: Third-Party Panels (the most common route)

Specialist healthcare panels such as M3, Sermo, Medscape, Doceree, and regional equivalents maintain large databases of HCPs willing to participate in surveys.

- Coverage is broad — M3 alone claims 3.5M+ HCPs globally
- Cost: typically charged per complete. Rates vary: $80–300 for a GP, $300–800 for a specialist, $1,000–2,000+ for a rare specialist
- Fraud risk: panel quality varies. Good firms run multiple fraud checks (open-end responses, timing, IP analysis, verification calls)
- Panel fees are a major OOP cost — often 30–60% of the total bid value
- In the bid: you list which panel(s) you will use, the cost per complete, and the total panel budget

Source 3: Professional Recruiters (for hard-to-reach audiences)

For rare specialties, senior KOLs (Key Opinion Leaders), payers, or HTA decision-makers, firms use specialist medical recruiters who phone-recruit from their own networks.

- Used when: rare disease, very senior HCPs (chiefs of medicine, HTA committee members), specific institutional settings
- Cost: recruiter fee per completed session. Typically $300-600/IDI for standard HCPs, $600-1,500+ for KOLs
- Slower: 3-6 weeks minimum for difficult-to-reach audiences
- In the bid: name the recruiter (or state 'specialist medical recruiter TBD'), provide estimated incidence, confirm feasibility

Source 4: Client-Provided Lists

Pharma clients sometimes provide a list of HCPs they want included — their own KOL list, or participants from a previous advisory board.

- Compliance check required: are these HCPs on any exclusion list? Are they competitors' employees?
- Consent: client may not have obtained research consent from these individuals — the firm must handle this
- In the bid: state that you can accommodate client-provided lists subject to consent and compliance checks

Source 5: Clinic/Site Recruitment

For patient research or some HCP studies, recruitment happens at clinical sites — hospitals, GP practices, oncology centres.

- Slower and more expensive but highest quality for patient studies
- Requires site agreements, ethics approvals (IRB/REC) — add weeks to the timeline
- In the bid: identify recruitment sites, flag ethics approval timeline, confirm patient eligibility criteria

What Goes Into the Bid About HCPs?

Do you reveal the HCP names in the bid?

No. You never share individual doctor names or contact details in the bid — this would be a serious data protection breach. What you DO share is: the universe size (how many eligible HCPs exist), the recruitment method, the incidence rate assumption, the feasibility rationale, and the shortlist structure (primary pool of X, backup pool of Y). The specific names are internal operational data.

What you share in the bid about HCP recruitment	What you keep internal
Universe estimate: '~2,400 eligible cardiologists in US meeting spec'	Individual HCP names and contact details
Recruitment source: 'Proprietary panel + M3 backup'	Panel member IDs or personal data
Feasibility rationale: '10% response rate assumption = 2,400 contacts needed for 200 completes'	Specific recruiter agreements or rate cards
Incentive amount: '$250 per complete, consistent with EFPIA fair market value'	Your database structure or sourcing methodology
Inclusion / exclusion criteria: 'Treat >= 5 eligible patients/month, no competitor affiliation'	Names of KOLs or advisory board members you plan to invite
Contingency plan: 'If primary panel is insufficient, we will supplement with [recruiter name]'	Anything that could be used to approach HCPs directly

PART 6 — What Happens After You Win? From Bid to Execution

Winning the bid is only the beginning. Now the firm must actually do what it promised. Here is the execution journey:

Phase 1 — Project Setup & Kick-Off (Week 1)

- Contract signed — SoW signed by both parties, PO received
- Project team briefed — everyone who worked on the bid hands over to the delivery team (or same people shift roles)
- Kick-off call with client — align on objectives, timeline, key contacts, communication cadence
- Project workspace set up — shared folder, project tracking tool, status report template

- Screener drafted — the short set of questions used to qualify respondents before the main study

Phase 2 — Instrument Design (Weeks 1–3)

This is where the survey or discussion guide is built. It is a highly iterative process:

- Research team drafts the survey questionnaire or discussion guide (DG)
- For quant: draft survey includes all question text, scales, logic/skip patterns, randomisation rules, stimulus files
- For qual: DG includes topic sections, probes, projective techniques, stimulus timing
- Internal review: senior researcher + compliance check (no leading questions, no adverse event language without consent)
- Client review round 1: client amends, adds questions, changes language
- Internal revisions: often 2-3 rounds before final sign-off
- Translation: if multi-country, all instruments translated + back-translated for accuracy
- Client approves final instrument: 'pencils down' — no more changes after this point

Phase 3 — Programming & Set-Up (Weeks 2–4)

- QUANT: Survey programmer codes the questionnaire into the survey platform (Confirmit, Qualtrics, Decipher, etc.)
- Pilot test: 3-5 internal pilots to check logic, timing, rendering across devices
- Client test link shared for review — client confirms final look/feel
- QUAL: Recruiter briefed, screener distributed, session scheduling begins
- IDI sessions set up: moderator guide finalised, video platform configured (Teams, Zoom, dedicated IDI platform)
- Stimulus prepared: concept boards, positioning statements, pricing scenarios formatted for screen

Phase 4 — Fieldwork / Data Collection (Weeks 3–10 depending on scale)

This is the most operationally intensive phase. For quant surveys:

- Soft launch: send to small sample (5–10% of target) first — check for any errors, timing issues, drop-off points
- Review soft launch data with client — confirm before full launch
- Full launch: send survey to full panel/contact list
- Pacing reports: daily or twice-daily report to client on completes by country/quota
- Quota management: as cells fill, close them. Redirect remaining respondents to underfilled cells.
- Fraud checks: daily — remove speeders (too fast), straight-liners (same answer every question), failed attention checks
- Field close: when all quotas are met or deadline is reached

For qual IDIs:

- Recruitment: recruiter contacts HCPs, screens them, schedules sessions (often 2-4 weeks of scheduling)
- Sessions conducted: moderator leads 45-90 min video interview per HCP
- Simultaneous note-taking: usually a second researcher observes and notes

- Recording + transcription: sessions recorded with consent, transcribed within 48 hours
- Interim debrief: after first 10-15 sessions, client debrief on emerging themes
- Field close: all sessions complete

Phase 5 — Analysis & Reporting (Weeks 8–14)

This is where the data becomes insights:

- QUANT: Data processing team cleans data, applies weights, builds SPSS/Q/Dapresy data file
- Tabbook created: cross-tabulation of every question by every key subgroup (country, specialty, etc.)
- Analysis plan executed: key drivers, segmentation model, conjoint utilities calculated
- Charting: analyst builds PowerPoint charts from the data
- Report written: Research Lead writes the narrative — not just 'what' but 'so what' and 'now what'
- QUAL: Analyst codes transcripts (open-end coding), identifies themes, builds quote bank
- Insight synthesis: cross-case analysis to identify patterns across all IDI participants
- Report/deck built: typically 50-150 slides for a full quant study; 30-80 for a qual study
- Internal QA: second researcher sense-checks data, charts, and conclusions
- Client review draft: client gets draft report, provides comments (typically 3-5 days)
- Final delivery: revised report + all deliverables sent
- Readout session: researcher presents key findings to client team (60-90 mins, often senior stakeholders)

PART 7 — How to Win a PMR Bid The Playbook

This section is about what separates a winning bid from a losing one. Based on how the industry actually works, here are the factors that matter most:

The 6 Things Clients Use to Choose a Firm

Selection criterion	Weight (typical)	What this means for your bid
Methodological quality and innovation	Very high	Your research design must be defensible, well-reasoned, and show real expertise. Clients show bids to their internal research team — it must pass scientific scrutiny.
Feasibility credibility	Very high	Clients have been burned by firms that promise 200 cardiologists in 3 weeks and deliver 120 in 6. Demonstrate exactly how you will recruit, with real universe numbers.
Understanding of the brief	High	Restate their objective in your own words, correctly and insightfully. If you can surface a nuance they didn't articulate, you demonstrate true expertise.
Relevant experience and team credentials	High	A case study in the same therapy area, or the same study type, counts for a great deal. The lead researcher's credentials matter.
Responsiveness and professionalism	Medium-high	How fast did you respond? Was the proposal well-formatted, error-free? Did you send a covering email? Did you follow up?

Selection criterion	Weight (typical)	What this means for your bid
Price	Medium	Price rarely wins alone. But being significantly more expensive than competitors without clear justification will lose you the bid.

The 5 Biggest Bid-Losing Mistakes

Mistake	Why it costs you the bid
Generic methodology section	If your 'approach' reads like it could apply to any study, the client assumes you didn't read their brief carefully.
Unrealistic timelines	Promising delivery in 8 weeks when it cannot be done in under 12 destroys credibility — especially with experienced clients.
Vague feasibility statements	'We have access to a large panel of oncologists' is not good enough. State your universe size, response rate assumption, and specific source.
Assumptions buried or missing	If your price is based on assumptions (15-min LOI, no stimuli translation), state them explicitly. Clients who later find out will never use you again.
Proposal errors	Number inconsistencies (n=200 in one place, n=300 in another), wrong client name, wrong therapy area — one error makes clients question everything.

The 5 Best Ways to Differentiate and Win

- **The client's RFP contains specific phrases — 'prescribing drivers', 'unmet need', 'patient centricity'. Use these exact phrases in your proposal. It signals you listened.** Mirror their language.
- **Not a whole extra module — just one smart methodological observation. 'We would suggest including a brief message recall module in Section 3, as it will help you assess the impact of your sales force messaging at no significant additional cost.' This demonstrates genuine expertise.** Propose one idea they didn't ask for.
- **Most firms say 'we will recruit 200 oncologists'. The best firms show a country-by-country, audience-by-audience grid with feasibility numbers. It is harder to produce — but impossible to argue with.** Show the sample plan in detail.
- **Clients buy people as much as firms. A named Research Director with 3 relevant case studies is more reassuring than a generic 'our experienced team'.** Include a named lead researcher — with a photo and relevant study list.
- **Offer Good (minimum n, lower cost), Better (recommended), Best (high confidence n). This gives budget-conscious clients a way to say yes to something, rather than rejecting a proposal because they can't afford the full version.** Price options, not just one number.

PART 8 — Competitor Landscape Who Lumina's Customers Compete With

Understanding the competitive landscape of healthcare PMR is important for Lumina in two ways: (1) our customers compete with these firms, so we need to understand their workflows; (2) some of these firms may eventually become Lumina customers themselves.

Tier 1: Global Giants (full-service, massive scale)

Firm	Scale & presence	PMR strengths	Known for
IQVIA	~85,000 employees, 100+ countries. Healthcare data + research + consulting.	Unmatched real-world data integration with primary research. Massive HCP panel. Regulatory expertise.	Combining claims data with primary research. Very strong in oncology, rare disease.
Kantar Health (now Inizio)	Large; spun out of Kantar, now part of Inizio group.	Brand tracking, segmentation, patient journey. Strong quant.	Large-scale trackers. Good global reach.
Ipsos Healthcare	Part of Ipsos (global top-5 research firm). Dedicated healthcare division.	Payer research, patient-centricity, quant and qual both strong.	Payer/market access. Strong in EU5 markets.
Syneos Health Research	Part of Syneos Health (CRO). Healthcare-only PMR.	Integration with clinical trial expertise. Patient recruitment.	Late-stage commercial research. US-heavy.
ZS Associates	Consulting + analytics + PMR.	Commercial strategy + primary research combined. Strong analytics.	Sales force effectiveness. Data-heavy engagements.

Tier 2: Mid-Sized Specialists (the most direct competitors to Lumina's likely customers)

Firm	Specialty	Known for
Cello Health Insight (now Inizio Evoke)	Healthcare brand strategy + PMR. UK/US focus.	Creative qual + quant blend. Strong therapy area expertise. Boutique feel.
The Lancet Group / Rare Patient Voice	Patient and rare disease panels.	Patient recruitment for rare conditions — unparalleled coverage.
Shapiro+Raj	US-focused healthcare MR.	Messaging and concept testing. Strong with medical device and pharma.
Hall & Partners Health	Brand and comms research in health.	Emotional and brand tracking. Strong creative qual.
Research Partnership	Independent, UK-founded, global reach. Mid-sized.	Deep therapy area expertise. Strong payer and HTA research. Respected for qual depth.
Forrester Research / Decision Resources (DRG)	Analytics + PMR hybrid.	Epidemiology + primary research. Very data-heavy.

Tier 3: Boutique Specialists (niche, often very high quality)

Many excellent boutique firms operate in narrow niches: rare disease only, payer/HTA only, patient experience only, a single therapy area (oncology, CNS, immunology). These firms often win against the giants on quality and relationship.

What All These Firms Have in Common — and Where Lumina Fits

Shared pain point	Why it exists	How Lumina addresses it
Bid process is almost entirely manual	No purpose-built tool exists for PMR bid automation. Every firm uses Excel + Word + email.	Lumina automates the bid loop end-to-end — from email parsing to proposal generation.
Inconsistent estimation between bid leads	Each senior researcher estimates differently. Margins are hard to predict.	Lumina's estimation engine enforces a consistent task library and multiplier logic.
No audit trail on bid decisions	'Why did we price it at $180k?' — nobody can reliably answer this 6 months later.	Every number, assumption, and decision in Lumina is evidence-linked and timestamped.
Handoff from bid to delivery is broken	Winning team hands over to delivery team verbally or via email. Scope drifts.	Lumina's HandoffPack + CRAFT integration creates a digital, locked scope transfer.
Reporting on bid operations is poor	Most firms cannot tell you their win rate, average bid turnaround time, or margin accuracy.	Lumina's analytics module tracks all of these metrics automatically.

Summary: What Makes PMR Bids Hard

The brief is vague • The audience is hard to reach • The timeline is tight

The methodology must be scientifically defensible • The pricing is complex • The compliance stakes are high

Lumina exists to make all of this systematic, fast, and auditable.